

[FACILITY_NAME_001] (GMCH), [LOCATION_002]
[FACILITY_NAME]

CASE SUMMARY: ADMISSION & CHEMOTHERAPY PROGRESS NOTES
REPORT ID: GMCH-[INCIDENT_REPORT_001]
DATE: [DATE_TIME_013]

[MEDICAL_RECORD_001] :
Patient Name: [PERSON_009] [PERSON_006]
[PERSON_009] /Sex: [DATE_TIME_012] / Male
Aadhaar No: [AADHAAR_NUMBER_001]
MRN (Hospital ID): [MEDICAL_RECORD_NUMBER]
Father's Name: [PERSON_008] [PERSON_006]
Mother's Name: [PERSON_008] [PERSON_006]
[PERSON_007] : [PERSON_008] [PERSON_006] [PHONE_NUMBER_001])
Address: House No. 224, Sector 15-C, [LOCATION_002] - [PINCODE_001]

ADMISSION NOTE ([DATE_TIME_011]):
[PERSON_008] [PERSON_006] presented with persistent low-grade fever and bone pain for [DATE_TIME_010] . [PERSON_003]
[PERSON_008] was previously seen at a local clinic where he was [PHI_DIAGNOSIS_002]
[PHI_DIAGNOSIS_002] . [PERSON_008] [PERSON_006] (father) reports that [PERSON_008] [PERSON_006] has been unusually lethargic. Upon
examination, [PERSON_008] [PERSON_006] showed signs of pallor and mild hepatosplenomegaly.

[PERSON_008] [PERSON_006] mentions that there is no family [PHI_DIAGNOSIS_001] [PERSON_008] [PERSON_006]
(mother) was also interviewed; she noted that [PERSON_008] [PERSON_006] had a normal birth history.
[PERSON_008] [PERSON_006] blood work showed a high WBC count ([DATE_TIME_009]) and [LAB_VALUE] ,
suggestive of Acute Lymphoblastic Leukemia (ALL).

PROCEDURE NOTE ([DATE_TIME_008]):
Bone marrow aspiration and biopsy performed on [PERSON_008] [PERSON_006] under conscious sedation.
Procedure was explained to [PERSON_008] [PERSON_006] , and written informed consent was obtained
from [PERSON_008] [PERSON_006] in the presence of a witness. [PERSON_008] [PERSON_006] tolerated the procedure
well. [PERSON_008] [PERSON_006] remained by [PERSON_008] [PERSON_006] side during the recovery phase.

CHEMOTHERAPY LOG - PHASE I (INDUCTION):
[DATE_TIME_007] : [CLINICAL_PARAMS] administered to [PERSON_008] [PERSON_006] . [PERSON_008] [PERSON_006]
showed no immediate hypersensitivity. [PERSON_008] [PERSON_006] was instructed on monitoring [PERSON_003]
[PERSON_008] for peripheral neuropathy symptoms.
[DATE_TIME_006] : [CLINICAL_PARAMS] given IM to [PERSON_008] [PERSON_006] . Nursing notes
indicate [PERSON_008] [PERSON_006] was cooperative. [PERSON_008] [PERSON_006] reported that [PERSON_008] [PERSON_006] had
one episode of vomiting post-infusion.

NURSING HANDOVER:
[PERSON_008] [PERSON_006] is currently stable in [HOSPITAL_BED_NUMBER] . [PERSON_008] [PERSON_006] vitals are being monitored every
[DATE_TIME_005] . [PERSON_008] [PERSON_006] is the designated bedside attendant. [PERSON_008] [PERSON_006] has been
provided with the pharmacy list for [PERSON_008] [PERSON_006] medications. [PERSON_008] [PERSON_006] will relieve

[PERSON_006] at [DATE_TIME_004] [DATE_TIME_003]. Please ensure [PERSON_003] MRN ([MEDICAL_RECORD_NUMBER]) is checked before any medication delivery.

DISCHARGE PLAN (PROVISIONAL):

Once [PERSON_003] completes the induction phase, [PERSON_003] will be monitored as an outpatient. [PERSON_006] has been given a [DATE_TIME_002] helpline number. [PERSON_003] must return for follow-up on [DATE_TIME_001]. Dr. [PERSON_002], Senior Consultant, has reviewed [PERSON_003] progress.